

Lower Lip Squamous Cell Carcinoma

Squamous cell carcinoma (SCC) of the lower lip typically arises in the setting of chronic sun exposure and often begins as a roughened papule within actinic cheilitis or as scaly leukoplakia, progressing slowly to a tumor nodule. Clinical features suggestive of evolving SCC include persistent lip chapping with localized scale or crust, a red and white blotchy atrophic vermilion zone, an indistinct or "wandering" vermilion border, and small fissures or ulcerations within an area of indurated actinic cheilitis.

Pain or altered sensation should prompt evaluation for possible perineural invasion, which is more common in advanced lesions of the head and neck, including the lip.

It may also present as a peritonsillar abscess when associated with metastatic spread or regional involvement.

Genital Squamous Cell Carcinoma

SCC of the vulva most commonly arises on the anterior labia majora and may present as a small warty nodule or an erosive erythematous plaque. These lesions may be asymptomatic but are more often associated with pruritus or bleeding. Precursor conditions include lichen sclerosus.

Cervical SCC is strongly associated with high-risk human papillomavirus (HPV) infection, particularly HPV type 16.

SCC of the scrotum typically begins as a small pruritic verrucous lesion that becomes friable as it enlarges.

Penile SCC usually occurs in uncircumcised males and is rare in Western countries but may account for up to 10% of cancers in regions with poor genital hygiene. A key precursor is erythroplasia of Queyrat, characterized by a velvety red plaque. Risk factors include lack of circumcision, history of genital condyloma, phimosis, and lichen sclerosus et atrophicus.

Although genital SCC was once reported following long-term psoralen and UVA (PUVA) therapy, this risk has been minimized by shielding the genital area during treatment, making such cases rare today.

Perianal SCC can also occur and may be associated with HPV infection or chronic inflammatory conditions.

Scar Squamous Cell Carcinoma

SCCs arising in scars typically develop decades after the initial injury, often presenting with skin breakdown, persistent erosion, or ulceration. These are most commonly found on the lower extremities at sites of chronic venous insufficiency or pyogenic granuloma ulcers.

Nodularity may develop gradually, but detection is often delayed due to concealment by surrounding indurated scar tissue. In cases arising within chronic sinuses, nodularity may be absent. New or increasing pain, drainage, or bleeding in a chronic scar should raise suspicion for malignant transformation and warrants biopsy.

Keratoacanthoma

(For full discussion, see Chapter 113.)